جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : اسماء فتحي عبدالفتاح محمد
المحافظة : الجيزه **الحي / المدينة :** الوراق **العنوان :** وراق الحضر
رقم البطاقة : 27510262102688 **رقم التليفون :** 01063075142 **تاريخ تسجيل الإستمارة :** 18-10-2026
العمر(تاريخ الميلاد) : 26-10-1975 **المهنة :** استاذ جامعي

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: اسماء فتحي عبدالفتاح محمد
National ID: 27510262102688

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

2007

جوستاف روسي

Patient's Name:	**asmaa fathy abd al fatah**
Date:	**2026-04-09**
Referred by:	Prof.Dr. ct ncap iv
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary masses or nodules.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

❖ **Stationary course since the last study dated 23-11-2025**

Dr.Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]



اسماء
فتحى عبدالفتاح محمد
٢٠ ش الجنينة-وراق الحضر
الوراق - الجيزه

٢٧٥ ١٠ ٢٦ ٢١ ٠٢٦ ٨٨

L88486394

٧ ٧ ٤

Name: اسماء فتحي عبد الفتاح محمد	
MR. : 2007	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

PREM RECURRENT LEFT BREAST LEFT SUPRACLAV MET TO LUNG ,BONE ER POSITIVE PR POSITIVE HER2 3+ HISTORY 2017 RT MRM ADJ HERCEPTIN CHEMO ARIMIDEX TILL 2022 METS REC TAXOL AND HERCEPTIN DP NAVELBEINE TYKERB TILL 9/2025 DP ST GEMZAR CARBOPLATIN WITH REACTION FROM CARBO SHIFTED TO PLATINOL CTS SD OF PUL NODULES BS 6/2025 PD OF STERNAL LESION FOR C5 D1 GEMZAR 2000 PLATINOL 70 D1 D8 FOR NEW BS

WT 90 HT 170 SA2 PT REC ZOMETA ON 13/1/2026 FOR BS NEW CTS AFTER THIS CYCLE FOR C6 D1,D8 GEMZAR2000 ,PLATINOL 70

pharmacy visit for C6D8 gemzar 2000mg / Platinol 70mg

Pharmacy Visit For Cycle 7 day 8 Gemzar 2000 + Platinol 70 mg Labs 7/3 ... accepted

c8 d1 gemzar 2000 cisplatin 70 d8 pharmacy pls ensure addition of gastric support + double antiemetic

clinical pharmacy note : for c8d8 gemzar 2000 mg only due to unavailability of cisplatin

cts 9/4/2026 sd for new bs for referral for tdm1 from ngo for c9 d1 d8 gemzar platinol

clinical pharmacy note : after consult dr dina for gemzar only due to unavilability of platinol

c10 d1 gemzar 2000 mg + cisplatin + supp ttt (ppi , granytrl)after labs C10 D8 gem >> pharmacy

for new cts bs for c11 d1 gemzar 2000 platinol 70 mg and 1 proph neupogen

for c12d1 gemzar/platinol, after labs, CTs pending pt still hasn't applied for tdm1 from NGO! d8 pharmacy

Pharmacy Note BSA: 2 For C12 D8 Gemzar 2000 + Platinol 70mg Labs 20/7 .. accepted

for c13 gemzar 2000 platinol d1 d8 granitryl ppis cetal and 1 proph neupogen new cts bs pending (approval of keytruda pending)

pharmacy visit: c13 d8 gemzar 2000mg -platinol 70mg

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

for c14d1 gem/cis, labs accepted TLC 3 ANC 1.7 PLT 108 , CT done without comparison > req, D8 pharmacy

Pharmacy Visit BSA: 2 FOr C14 D8 Gemzar 2000 + Platinol 70 mg Labs 31/8 ... accepted

٢٧٥١٠٢٦٢١٠٢٦٨٨ ٢٠٢٥/٠٤

حاصلة على دبلوم تجارة

أنثى مسلمة متزوجة

ممدوح ابراهيم احمد طايع

البطاقة سارية حتى ٢٠٣٢/٠٤/٠٥

١

جوستاف روسي انترناشونال مصر

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 25/ 8 /2025 G

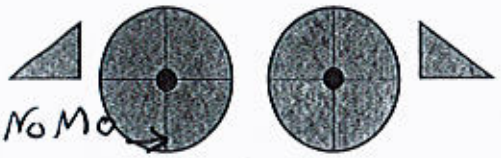
Name	اسماء فتحي عبد القناع	MRN	1829/22
		ID	[illegible]
Age	49	Phone	01063075142
Menopausal status		Marital status	married
NRS SCORE		Family history	+ve F.H
BMI		REASON OF MDT	
CO morbidity	DM- HTN		

Clinical Data

BX: IDC GII

ER:	PR:	HER 2:	Ki67:
+ve	+ve	+ve	30%

R Ps 1 L



Sonomammography:
8/2016 → diagnosed Breast Cancer T4bN0M0 → relieved NACT AC x4, taxol x4 → lt MRM + reconstruction + AI + Zoladex NM from 6/2017 → 3/2

Mets Workup: Perth ⇒ Herceptin x 17

3/2022 ⇒ lt S.C.LN ⇒ Bx ⇒ triple +ve, relieved Herceptin + 12 (taxol 6 then maintance Herceptin I.V + aromasine

10/23 ⇒ Bone SCan ⇒ newly developed bone mets

Recommendations:
MDT ⇒ for xeloda + tykerb → Patient relieved xeloda x6 cycles (لا يجد متوفر + لا تتحمله بروفه)

shifted in the clinic to navelbine oral ⇒ Patient continued on tykerb, navelbine → last navelbine (3/2025) ثم توقف القرار ← last (5/2025) توقف انتا تترب؟ → then relieved only 1 cycle

MDT OUTCOME & PLAN Bone SCan 6/2025 ⇒ Progressive Course. off tykerb (7/8/2025)

CTCAP 7/2025 ⇒ Progressive Course

for 3rd line Cemcer/ Carbo

د. نرمين عيسى

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

مره
مدام
ثولوجى

Patient Name :	اسماء فتحى عبدالفتاح محمد	**Pathology No :**	**2347/22**
Sex :	**Age :** 46	**Hospital No.**	**16402/16**
Clinical Data:	Core biopsy from Lt. supra-clavicular lymph node. History of Lt. MRM, post CTH (		
Department :	مريض عياده خارجيه وحده جراحه - د	**Referring Dr. :**	

Electronically Authorized by: Mohamed Yousri Ahmed

Gross :

Three cores 0.5-1 cm.

Histologic :

Core biopsy shows metatatic adenocarcinoma formed of irregular solid groups of malignant epithelial cells with focal ductal differentiation. Anaplasia and mitosis are moderate. Separating stroma is fibrotic. In view of previous history, metastatic duct carcinoma of breast origin is the main possibility. GATA-3 is positive. ER is positive (+++). PR is positive (+). HER2 is positive (score 3+).

Diagnosis Lymph node,Biopsy:Metatsatic adenocarcinoma, consistent with breast origin. ,Grade 2.

Category Malignant tumour, Metastatic Recurrence

Pathologists :

Dr. Mona Ali Abdel Fatah Sakr

Dr.

Dr. Mohamed Yousri Ahmed

REPORT WAS SIGNED ON 01/03/2022

Verbal Revision Mohamed Yousri Ahmed

Typist sana'a abdel aziz ismael

Date of Issue : 24/05/2022

FATHE ABDULLFATTAH, ASMAA Patient ID: 116016402 DOB: 26-Oct-1975 Date: 25-Jun-2025
Patient Lastname: Fathe Abdullfattah
Patient Firstname: Asmaa
HIS Patient ID: 116016402
RIS Patient ID: 146048
Date of Birth: 26/10/1975

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========

99mTc MDP Bone Scan Report 26/6/2025
Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

Compared to the previous bone scan dated (13/8/2024), the current study reveals:
" Progression of the previously detected active tracer uptake seen at the sternal body keeping with metastatic osseous deposit.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both Kidneys are visualized.
Conclusion: -
Follow up bone scan of a known case of breast cancer shows:
" Progressive course of the previously seen active sternal osseous lesions; likely metastatic, to be correlated with CT cuts.
" No other definite active osseous lesions detected.
Much Obliged
Areej Atran
Mohamed Abdeltawab, MSc

========== REPORT TEXT ENDE ==========

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name: Asmaa Fat'hy Abd Al Fattah Mohamed	Date : 26/07/2025
Exam : CT CAP(CT Chest Abd. Pelvis)	Age : 49 Year(s)
Medical ID:	Pat. ID: 32762

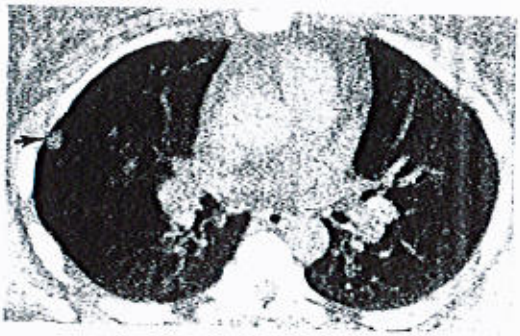
C.T. SCAN OF THE CHEST WITHOUT CONTRAST / HRCT

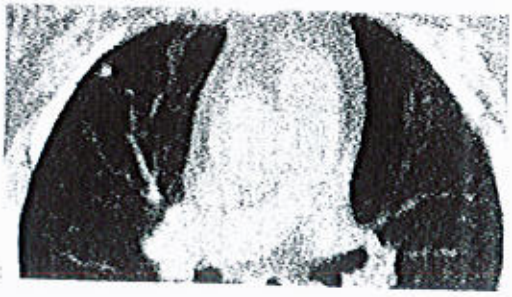
Axial contiguous thin cuts were taken through the chest without I.V. contrast injection.

Follow up study as compared to previous report dated 17/04/2025 ,the current study reveals:

FINDINGS:

- **Progressive course** as regards the size and number of the previously noted multiple scattered pulmonary nodules ranging in size from 3to 11 mm, being more on the right side.
- **Denovo** mild diffuse moasic appearnce denoting small air way disease





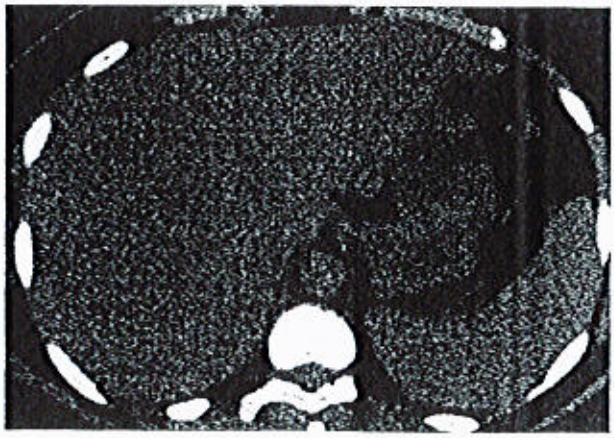
Much obliged,

Mona Ali,MD

CT SCAN OF THE ABDOMEN AND PELVIS WITHOUT CONTRAST.

TECHNIQUE: Contiguous helical axial images of the abdomen and pelvis without administration of intravenous contrast.

FINDINGS:



- ***The liver is mildly enlarged with homogenous decreased hepatic attenuation.*** No focal lesions or contour irregularities. There is no evidence of intrahepatic or extra-hepatic biliary dilatation.
- The gall bladder is within normal limits.
- The spleen, the pancreas, adrenal glands and both kidneys are within normal limits.
- No evidence of abdominal aortic aneurysm.

- No evidence of abnormal masses or consolidative patches regarding the non contrast study.
- No CT evidence of abnormal areas of crazy paving.
- **Small aortopulmonary lymph nodes are noted.**
- **Few bilateral axillary lymph nodes.**
- Patent tracheo bronchial tree with no abnormal bronchial dilatation.
- No pleural or pericardial effusion.
- Average size of the heart is noted.
- **Still noted** mild diffuse increase in right breast skin thickness.
- The included lower neck cuts are unremarkable
- **Still noted** sclerosis of the sternal body .



IMPRESSION:-

Follow up study revealing :

- **Progressive course as regards the size and number of the multiple scattered pulmonary nodules.**
- **Denovo mild diffuse moasic appearnce denoting small air way disease**
- **Stationary course as regards the mild diffuse increase in right breast skin thickness.**
- **Stationary course as regards the sternal body sclerosis.**

- The visualized portion of the bowel is unremarkable.
- Normal appearance of the urinary bladder with normal wall thickness and no calculi.
- Normal appearance of the rectum, and rectosigmoid junction.
- No pelvic fluid collection.
- ***Few small para-aortic lymph nodes***
- ***Mild diffuse dorso-lumbar spondylodegenerative changes***

IMPRESSION:

- **Mild diffuse fatty hepatomegaly for correlation with hepatic and lipid profile .**
- **Few small para-aortic lymph nodes**
- **Mild diffuse dorso-lumbar spondylodegenerative changes**

Much Obliged,

Mona Ali,MD

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name: Asmaa Fathy Abdel Fattah Mohammed

Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Examination:

■ Computed tomography of the chest, abdomen and pelvis

Technical data:

- 5 mm CT scans of the abdomen and pelvis with IV contrast

Clinical data: known case of left right breast cancer
Comparison: in correlation to the available study 9-4-2026
Findings:

- **Chest findings:**
- **Still seen sternum heterogeneous sclerotic densities**
- **Progressive course in numbers of the bilateral lungs multiple small nodules mainly peripheral, subpleural and peri fissural in location ranging 4-12mm**
- Clear rest lungs fields
- No hilar or mediastinal masses or sizable LNs.
- Normal appearance of the tracheobronchial tree.
- No evidence of pleural effusion or thickening.
- Average cardiac size, no pericardial effusion.
- **Abdomen and pelvis findings:**
- The liver shows enlarged size (RT lobe length 18 cm) with smooth contours, homogeneous hepatic parenchyma density, and no evidence of focal lesions.
- No intra hepatic biliary dilatation.
- Normal caliber and homogenous enhancement of the portal vein.
- The spleen shows normal size. No evidence of focal lesions.
- Normal CT appearance of the gall bladder, pancreas and adrenal glands
- Both kidneys are of normal site,
  mildly reduced size left kidney with lobulated outline
  RT = 11.5x6.5 cm and parenchymal thickness 2.2 cm
  RT = 9.6x5 cm and parenchymal thickness reaches down to 1.4 cm
  no renal stones, back pressure, cysts or masses bilaterally.
- The urinary bladder shows average distention, no wall thickening, n stones, diverticulae or masses.

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name: Asmaa Fathy Abdel Fattah Mohammed

C-V

Date: 2026-08-24
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- No suspicious abdominal or pelvis enlarged LNs
- No CT evidence of intestinal obstruction
- No pneumoperitoneum
- No free or localized abdominal or pelvis fluid collection

Opinion:

- **Progressive course of pulmonary nodules**
- **Stationary rest of study**

Regards, Dr. Ahmed Rezk, MD.

Ahmed

Ahmed